

CONSULTATION NOTE — CARDIOLOGY OPD

Patient	Ravi Kumar	Date	18-Mar-2025
Age/Sex	58 Years / Male	Visit Type	Follow-up
Consultant	Dr. Kavitha Menon, DM Cardiology	BP	138/88 mmHg
Weight	74 kg (prev: 76kg)	Pulse	72 bpm, regular

SUBJECTIVE

Patient reports improvement in exertional dyspnoea. Chest tightness episodes reduced in frequency — 2 episodes in past month vs daily at admission. Pedal oedema resolved. Fatigue improved. Blood sugar readings at home: FBS 140–170 mg/dL range. Compliant with medications.

OBJECTIVE

- BP: 138/88 mmHg (right arm, sitting) — improved but target not yet met (< 130/80)
- HR: 72 bpm, regular rhythm
- SpO2: 98% room air
- Weight: 74 kg (lost 2 kg since discharge — positive)
- Pedal oedema: Absent
- Chest auscultation: Clear air entry, no murmurs

ASSESSMENT

1. T2DM — improving, HbA1c 8.1% (was 9.2%). Still above target.
2. Hypertensive Heart Disease — BP improving but not at target. LVH on Echo.
3. Dyslipidaemia — LDL improving (142 → 118). Still above target of < 70 mg/dL for cardiac patients.
4. CKD Stage 2 — creatinine trending up. Microalbuminuria detected on latest labs. CONCERNING.
5. Mild anaemia — likely iron deficiency given microcytic picture.

PLAN

- Increase Telmisartan from 40mg to 80mg OD — for BP and renal protection
- Add Dapagliflozin 10mg OD — SGLT2 inhibitor for diabetes AND renal protection
- Refer to Nephrology for microalbuminuria and CKD monitoring
- Iron studies, peripheral smear — for anaemia workup
- Increase Rosuvastatin to 40mg — LDL target < 70 for cardiac patients
- **DO NOT USE Penicillin or any beta-lactam antibiotics — anaphylaxis risk**
- Repeat labs in 6 weeks. Next cardiology review in 3 months.